INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Trastuzumab emtansine (T-DM1) · metastatic

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
21 days
See the active treatment phase

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00206
Version 5

T-DM1 3.6 mg/kg intravenous (IV) every 21 days, adjusted if required, until progression or unacceptable toxicity. This metastatic course has no fixed 14-cycle endpoint; the postoperative early-breast regimen is a different card.

Treatment stages

01 **Current HER2 treatment**
Every 3 weeks

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Call promptly for unusual bleeding, yellow eyes / skin, worsening numbness or new cough / breathlessness.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

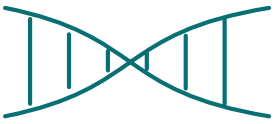
The medicine in this card

T-DM1 links HER2 targeting to a chemotherapy payload to help control previously treated advanced breast cancer. Prior trastuzumab and a taxane, plus the full NCCP prior-treatment or early-relapse criteria, must be checked.

HER2-DIRECTED ANTIBODY WITH A CHEMOTHERAPY PAYLOAD

Trastuzumab emtansine (T-DM1)

Kadcyla · antibody–drug conjugate



HER2 targeting carries DM1 into cancer cells.

The antibody attaches to HER2. After the complex enters a cell, its DM1 payload disrupts structures needed for cell division. This is a single linked medicine, with its own dosing and safety rules.

Attach to HER2 → Enter the cell → Release DM1 to disrupt division

What to watch for

Report bleeding, numbness, jaundice, new cough or breathlessness. Keep blood and heart checks.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Why this treatment?

This treatment is for HER2-positive breast cancer that cannot be removed by surgery or has spread, after specified earlier treatment with trastuzumab and a taxane. T-DM1 carries a chemotherapy medicine attached to an antibody that targets HER2.

Why this medicine rather than another HER2 treatment?

The decision depends on the treatments already received, when the cancer returned or changed, organ function, side effects and other suitable options. T-DM1 is different from trastuzumab alone and from trastuzumab deruxtecan (T-DXd). Ask your team to explain why it fits your current plan.

Your own decision

Ask which cancer features, earlier treatments and health factors led to this recommendation, what alternatives were considered, and when the plan will be reviewed.

Your treatment aim

Setting and duration

T-DM1 3.6 mg/kg intravenous (IV) every 21 days, adjusted if required, until progression or unacceptable toxicity. This metastatic course has no fixed 14-cycle endpoint; the postoperative early-breast regimen is a different card.

Medicine identity

T-DM1 is a single antibody–drug conjugate. It is not interchangeable with trastuzumab alone or trastuzumab deruxtecan.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Where this treatment fits

Trastuzumab emtansine (T-DM1) · metastatic

21-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

EVERY 3 WEEKS

Current HER2 treatment

Repeated cycles

T-DM1 3.6 mg/kg IV every 21 days, adjusted if required, until progression or unacceptable toxicity. This metastatic course has no fixed 14-cycle endpoint; the postoperative early-breast regimen is a different card.

Early and metastatic pathways have different aims and endpoints. The team confirms the applicable plan.

BASELINE → TREATMENT → FOLLOW-UP

When your checks happen

Cycle examples assume no delays and count from the first dose of this schedule. If switching route or continuing previous HER2 treatment, preserve the existing echo due date. Tests are scheduled by elapsed time and clinical need; the team owns booking and result review. NCCP remains 12-weekly during treatment, including maintenance; a longer interval needs an explicit individual specialist plan. Metastatic T-DM1 continues according to benefit and tolerance; the early-breast 14-cycle course does not apply.

Check	Before first dose	Early visits	Week 6 · around C3	Week 12 · around C5	Later / after stopping
Heart pumping test · echo / multigated acquisition (heart pumping) scan (MUGA)	Baseline before starting	Earlier if symptoms or cardiac concern	Not routinely repeated just because week 6 arrives	Repeat at 12 weeks; earlier checks may be needed	Every 12 weeks during treatment. Review cardiac follow-up when treatment changes.
Blood tests	Baseline full blood count (FBC), renal and liver profile	Before each cycle	Repeat before this cycle	Before every cycle	Every cycle

Check	Before first dose	Early visits	Week 6 · around C3	Week 12 · around C5	Later / after stopping
electrocardiogram (ECG) / rhythm	ECG if clinically indicated; baseline echo / MUGA required.	Symptoms or clinical indication	Clinical plan	Clinical plan	Do not substitute ECG for left ventricular ejection fraction (heart pumping measurement) (LVEF) monitoring
Cancer scans / markers	Record the disease sites and comparison imaging	Assess symptoms at every visit	Scan if due or indicated	Response review on the individual plan	Metastatic response scans commonly every 2–4 months; markers supplement the full assessment.
dual-energy X-ray absorptiometry (bone density) (DEXA) / bone health	Assess bone-health need from the individual treatment and fracture risk	Not an automatic T-DM1 test	Timing depends on risk and results	Separate from echo monitoring	Any endocrine treatment or bone protection requires its own oncology plan.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00206 v5 · official regimen · ESC 2022 cardio-oncology guideline · cardiac surveillance and risk · ESMO 2024 early breast cancer guideline · treatment and follow-up · NICE NG101 · bone health and follow-up · ESMO metastatic breast cancer guideline · response assessment · ASCO · biomarkers in metastatic breast cancer · EMA Kadcyla product information · metastatic toxicity table

THE WHOLE TREATMENT JOURNEY

Trastuzumab emtansine (T-DM1) · metastatic: the whole pathway

Previously treated HER2-positive unresectable locally advanced or metastatic breast cancer

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before treatment	First cycles	Around 12 weeks	Continuing treatment	Completion / change
Cancer medicine NCCP 00206	Confirm setting, prior doses and formulation	T-DM1 IV every 3 weeks	Review benefit, symptoms and heart function	Continue while beneficial and tolerated	Review cancer control and tolerance; agree continuation or a change.
Surgery METASTATIC	Local procedures only if indicated	No automatic curative surgical sequence	Coordinate procedures with oncology	Symptom / site-specific decisions	Local follow-up as indicated
Radiotherapy EARLY · METASTATIC	Assess indication and planned site	Timing depends on the clinical need	Agree any hold / continuation with radiation oncology	radiotherapy (RT) is site / symptom directed. Agree any treatment hold with both teams.	Radiation effects and surveillance follow the local plan
Blood tests NCCP 00206	Baseline FBC, renal and liver tests	Before every cycle	Before every cycle, including around C5	Before every cycle	Review abnormalities and the next treatment plan
Heart checks NCCP 00206 · SMPC	Baseline echo / MUGA; cardiac assessment	Symptoms can trigger earlier checks	Echo / MUGA every 12 weeks; around C5 for uninterrupted 21-day cycles	Repeat every 12 weeks; 4–8 weeks may be needed with impairment	Follow-up depends on cardiac findings and the next treatment plan.
Scans / markers EARLY · METASTATIC · MARKERS	Define baseline sites and scan purpose	Review symptoms at each visit	Response imaging when due; exact timing agreed for the known disease sites.	Continue the individual metastatic response-assessment plan.	Response scans commonly every 2–4 months; individualise. Markers supplement clinical review and imaging.
Endocrine / bone health EARLY · BONE	Check hormone receptors, previous treatments and bone-health needs	Any endocrine treatment requires a separate oncology plan	DEXA only if indicated by endocrine treatment or fracture risk	Do not add another anticancer medicine from this component card	Continue the individual bone and supportive-care plan

What each check is for

1



Blood tests

FBC, renal and liver profile before every cycle

Results help determine whether treatment can proceed.

Ask / act: The team confirms booking, results and next dose.

2



Echo / MUGA

Echo / MUGA every 12 weeks; around C5 for uninterrupted 21-day cycles

Heart function can change without symptoms.

Ask / act: Keep the booked scan; report symptoms between scans.

3



Cancer imaging and markers

Response imaging commonly every 2–4 months, adapted to sites, disease course and treatment. A new symptom can need earlier imaging. Tumour markers may supplement assessment, never determine a treatment change alone.

Use the result for a specific clinical decision.

Ask / act: Ask which scan is planned and who reviews it.

4

Booking and result ownership

At initiation, each review and handover

A recommended test can be missed without a named owner.

Ask / act: Record booking owner, result reviewer, due window and escalation route.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Your practical treatment plan

STEP 1

Before your visit

Confirm the dose, symptoms, required blood results and echo due window.

STEP 2

Access and administration

A cannula, peripherally inserted central catheter (PICC) or implanted port may be used according to the treatment plan. A port insertion is a separate procedure.

STEP 3

Treatment and observation

First infusion over 90 minutes, then observe for at least 90 minutes. Later infusions may take 30 minutes if tolerated, with at least 30 minutes observation.

STEP 4

After the visit

Contact the unit to rearrange a missed dose; the next schedule preserves a 3-week interval. This medicine does not routinely require a take-home pump.

Port insertion and removal

The line team explains placement, dressing care and when normal activities can resume. A port is accessed with a special needle during treatment. Removal of the implanted device is a separate planned procedure when no longer needed; do not remove or adjust it yourself.

PICC care and urgent line symptoms

A PICC usually needs regular dressing and flushing appointments even between treatments. Fever, redness, swelling, pain, leakage or damage requires prompt advice. Your hospital supplies the exact care instructions.

Before treatment and between visits

The team should arrange and review the tests.

Blood checks

FBC, renal and liver profile before every cycle

Cardiac checks

Echo / MUGA every 12 weeks; around C5 for uninterrupted 21-day cycles. Baseline cardiac assessment and LVEF are required. ECG is guided by the cardiac / clinical assessment; NCCP does not prescribe a separate fixed-cycle ECG timetable for T-DM1.

After stopping

Agree follow-up of cardiac, liver, platelet and nerve effects at any treatment change; no universal post-T-DM1 scan calendar is assigned here.

The wider pathway

Coordinate cancer medicines with local treatment and follow-up.

Scans and markers

Response imaging commonly every 2–4 months, adapted to sites, disease course and treatment. A new symptom can need earlier imaging. Tumour markers may supplement assessment, never determine a treatment change alone.

Radiotherapy relationship

Radiotherapy is used for a defined site or symptom when indicated. The oncology and radiation teams agree whether this medicine continues or pauses; do not infer a concurrent schedule from this card.

Bone health

If an aromatase inhibitor or ovarian suppression is planned, assess fracture risk and DEXA need. Repeat timing depends on results and bone treatment; this is not an automatic HER2-therapy scan.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Day 1 → Day 21: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
IV treatment	 **Day 1** Planned dose
Administration reaction	 **Day 1** During / after visit; report promptly
Fatigue / bowel changes	 **Days 1–21** Timing and severity vary
Heart / breathing symptoms	 **Days 1–21** Report on any day
Bleeding / bruising	 **Days 1–21** Low platelets can occur
Numbness / tingling	 **Days 1–21** Can build over treatment

No date entry needed: This is an orientation map; it does not calculate a personal appointment calendar.

Companion medicines: If another cancer medicine is prescribed, its side effects, tests and supportive medicines need its own plan.

Delayed treatment: Blood tests and echoes follow clinical due windows. A postponed dose does not automatically postpone monitoring.

Why these bands stay broad: Treatment days show the planned schedule. Symptom bands are broad possibilities, not measured risk curves or guaranteed safe days. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00206 v5 · official regimen ↗

There are no guaranteed "safe days". New breathlessness, chest pain, collapse, severe allergic symptoms or sudden serious illness need urgent assessment on any day.

Between-treatment notes

Day 1–21: Your reference cycle

T-DM1 3.6 mg/kg IV every 21 days, adjusted if required, until progression or unacceptable toxicity. This metastatic course has no fixed 14-cycle endpoint; the postoperative early-breast regimen is a different card. The team confirms each dose after the relevant checks. Contact them about a missed visit or new symptoms.

Support around the medicines

1 **Support for this medicine**

T-DM1 has low emetogenic risk; review the low-risk antiemetic draft and individual contraindications.

2 **Blood and heart appointments**

FBC, renal and liver profile before every cycle. Echo / MUGA every 12 weeks; around C5 for uninterrupted 21-day cycles.

3 **Other treatments and bone health**

Radiotherapy or local procedures are used only if indicated. Any endocrine or other systemic treatment needs a separate oncology plan. Bone-health checks depend on the individual risk and treatment context.

4 **Pregnancy / contraception**

Discuss pregnancy, fertility and contraception before treatment. Report pregnancy promptly; do not breastfeed during treatment or for 7 months afterwards. The team confirms the medicine-specific contraception duration.

Questions worth asking

When is my next echo?

Echo / MUGA every 12 weeks; around C5 for uninterrupted 21-day cycles. Existing HER2 exposure and delays affect cycle numbering; the team should give the actual due window.

Does a tumour marker replace a scan?

No. In metastatic disease it may add information to symptoms and imaging. It should not alone determine treatment changes. In symptom-free early disease it is not routine recurrence screening.

Why was this treatment chosen for me?

This treatment is for HER2-positive breast cancer that cannot be removed by surgery or has spread, after specified earlier treatment with trastuzumab and a taxane. T-DM1 carries a chemotherapy medicine attached to an antibody that targets HER2. The decision depends on the treatments already received, when the cancer returned or changed, organ function, side effects and other suitable options. T-DM1 is different from trastuzumab alone and from trastuzumab deruxtecan (T-DXd). Ask your team to explain why it fits your current plan.

Try explaining the plan back: What is the purpose of your treatment? Which heart or blood test is due next, and which team arranges and reviews it?

Trastuzumab emtansine (T-DM1)

- Low platelets and bleeding
- Liver injury, rarely serious
- Numbness or tingling
- Heart pumping-function reduction
- Rare serious lung inflammation
- Fatigue, nausea and infusion reactions

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Metastatic T-DM1

Active medicines: Trastuzumab emtansine (T-DM1)

Emetogenic risk: Low

Medicine	Dose / route	When
Ondansetron	8 mg orally	On treatment day Review QT, constipation and hepatic dosing; reconcile any steroid already prescribed
Metoclopramide	10 mg orally up to three times daily as needed	Subsequent days: duration to be confirmed Assess contraindications, renal function and interactions

Additional phase-specific support

- T-DM1 has low emetogenic risk; review the low-risk antiemetic draft and individual contraindications.
- First infusion over 90 minutes, then observe for at least 90 minutes. Later infusions may take 30 minutes if tolerated, with at least 30 minutes observation.
- Contact the unit to rearrange a missed dose; the next schedule preserves a 3-week interval.
- Companion chemotherapy or endocrine drugs need their own supportive review; this draft covers the named HER2 component only.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Trastuzumab emtansine (T-DM1) · metastatic

T-DM1 3.6 mg/kg intravenous (IV) every 21 days, adjusted if required, until progression or unacceptable toxicity. This metastatic course has no fixed 14-cycle endpoint; the postoperative early-breast regimen is a different card.

Source reconciled; independent clinical, pharmacy and patient review pending. Selected effects; not a complete medicine leaflet.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Breathing

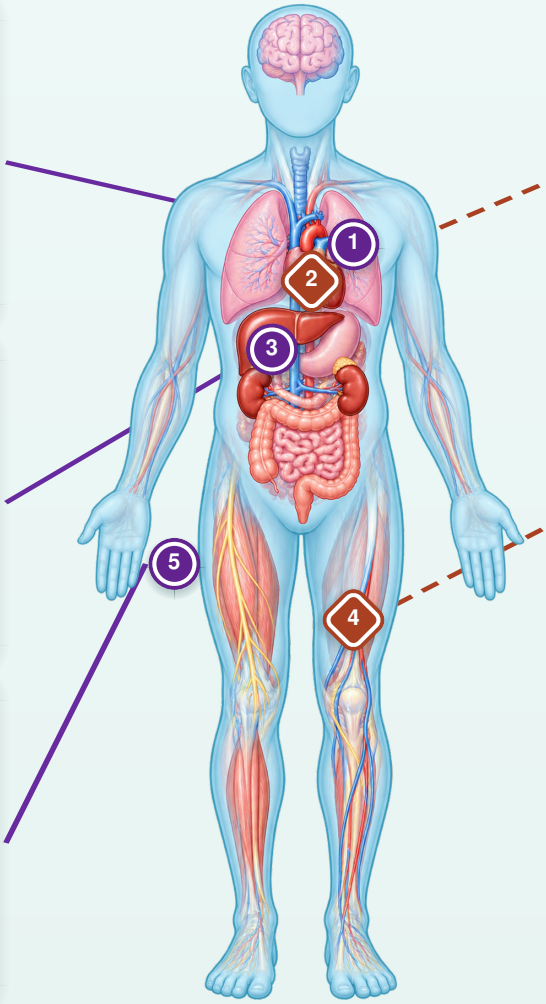
Trastuzumab emtansine (T-DM1)

New cough or breathlessness: seek prompt assessment.

2 Heart

Trastuzumab emtansine (T-DM1)

Keep echo checks. Report swelling, chest symptoms, fainting or rapid weight gain.



3 Liver

T-DM1

Keep liver blood tests. Report yellow eyes / skin, dark urine or significant abdominal symptoms.

4 Platelets / bleeding

T-DM1

Report unexplained bruising or bleeding; heavy or persistent bleeding needs urgent help.

5 Nerves

T-DM1

Report numbness or tingling, especially if it affects walking or hand use.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Fever, shivers or suddenly unwell: contact oncology immediately. Severe chest pain, breathing difficulty, collapse or seizure: 112 / 999.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **NCCP 00206 v5 · official regimen ↗**
   https://healthservice.hse.ie/documents/6547/206_Trastuzumab_Emtansine_21days.pdf
2. **ESC 2022 cardio-oncology guideline · cardiac surveillance and risk**
   https://academic.oup.com/eurheartj/article/43/41/4229/6673995
3. **ESMO 2024 early breast cancer guideline · treatment and follow-up**
   https://www.annalsofoncology.org/article/S0923-7534(23)05104-9/fulltext
4. **NICE NG101 · bone health and follow-up**
   https://www.nice.org.uk/guidance/ng101/chapter/Recommendations
5. **ESMO metastatic breast cancer guideline · response assessment**
   https://www.annalsofoncology.org/article/S0923-7534(26)00902-6/fulltext
6. **ASCO · biomarkers in metastatic breast cancer**
   https://ascopubs.org/doi/10.1200/JCO.2015.61.1459
7. **EMA Kadcyla product information · metastatic toxicity table**
   https://www.ema.europa.eu/en/documents/product-information/kadcyla-epar-product-information_en.pdf
8. **eviQ · implanted port patient information**
   https://www.eviq.org.au/p/507
9. **eviQ · peripherally inserted central catheter (PICC) patient information**
   https://www.eviq.org.au/p/543
10. **HSE · chemotherapy side effects and urgent advice**
    https://www2.hse.ie/conditions/chemotherapy/side-effects/
11. **NCCP NCIS antiemetics v8 (2025), Table 1**
    https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
12. **NOGG · section 7, aromatase inhibitors and fracture risk**
    https://www.nogg.org.uk/full-guideline/section-7-strategies-management-osteoporosis-and-fracture-risk